GYN QUICK REFERENCE GUIDE

Fallopian Tubes

- Sterilization- complete cross section(s) and entire fimbria perpendicularly sectioned
- Ectopic- 3 blocks of POC or suspected area and entire fimbria perpendicularly sectioned
- Incidental tubes without abnormalities-submit three cross sections of each tube (proximal, mid and distal), in one cassette for each tube. Submit entire fimbria perpendicularly sectioned
- Lesional- 3 sections to include grossly uninvolved mucosa and entire fimbria perpendicularly sectioned

Ovary mass

- One section per centimeter of tumor including 1 to surface
    - IF mucinous 2 sections per centimeter
- Fallopian tubes- entire fimbria perpendicularly sectioned and representative cross sections

Omentum

- WITH Nodule
    - All less than 2 cm- 2 cassettes of *different* nodules
    - At least 1 larger than 2 cm- submit 2 sections of largest
- WITHOUT nodule- 5 cassettes with 2 sections each

Lymph node

- Submit extra fibroadipose for each part following lymph nodes up to 10 total cassettes in addition to the LNs found

AUB

- NO lesions (polyp, leiomyoma)
    - Submit entire endometrium
    - WITH lesions (polyp, leiomyoma)
    - Do NOT submit entire endometrium

Cervix

- Cervical cancer- tumor present- 3 sections (or 1/cm) with greatest depth of invasion
- Unoriented top hat- no inking required

Lynch Syndrome

- Entire tubes and ovaries in 4-6 cassettes (If >6 casettes, submit representative ovaries)
- Submit entire endometrium

History of breast CA

NOT BRCA or other germline mutation

- entire tubes and representative ovaries in 4-6 cassettes do NOT submit entire endometrium

History of BRCA or other germline mutation- Risk Reducing

- Submit ENTIRE endometrium
- Submit ovaries and fallopian tubes per **SEE FIM protocol** (below)

SEE FIM PROTOCOL

OVARIES:

- Handle the ovaries as little as possible; we're looking for subtle changes in the surface epithelium, which is easily disrupted by lots of touching
- Section the ovaries at 2-3 mm intervals PERPENDICULAR to the long axis and submit entirely

FALLOPIAN TUBES:

- Detach fimbriated end of fallopian tube and section at 2-3 mm intervals and submit entirely
- Section the fallopian tube 2-3 mm intervals and submit entirely
- Submit the right and left sides in toto in separate cassettes

Leiomyoma

- <5cm without evidence of necrosis or hemorrhage: 1 section in 1 cassette
- All larger than 5cm, submit up to 5 cassettes representatively sampling the lesions
    - Submit additional sections from lesions with grossly visible degeneration, hemorrhage, cysts, or necrosis
- If hysterectomy/myomectomy is fragmented (no recognizable intact masses)
    - 1 section/cassette per 1 cm, up to 10 cassettes

POC/ Placenta

- Do NOT cut/submit heart
- Placenta
    - One needed at cord insertion site
    - Sections should be CENTRAL
    - History post-partum hemorrhage- 2 cassettes with 1-2 sections of superficial maternal surface to look for accreta to include disrupted areas
    - IUFD always submit 1 cassette of cords, 1 of membranes, and 8 parenchyma
    - True knot- submit proximal to and distal to in 2 labeled cassettes and take photo
    - Forked cord- distance to disc edge
    - Lesions- submit multiple if different appearances and include at least 1 normal parenchyma (add blocks if needed)